

****Medical Report****

****History****

The patient presented with a one-week history of intermittent central chest tightness, described as a squeezing sensation lasting 5–10 minutes. The pain was triggered by physical exertion, such as climbing stairs or walking quickly, and was relieved by rest. There was no radiation of pain. The patient also reported shortness of breath on exertion. There was no prior history of cardiac disease. The patient has a 20-pack-year smoking history (1 pack/day) and a family history of myocardial infarction in the father at age 60.

****Physical Examination****

Vital signs were as follows: blood pressure 120/80 mmHg, pulse 72 bpm, temperature 37 °C, and SpO 99%. Cardiovascular examination revealed regular heart sounds with no murmurs. Respiratory examination showed clear lung fields. There was no peripheral edema. A 12-lead ECG was performed and showed no acute ischemic changes.

****Assessment****

The patient's symptoms of exertional chest tightness and dyspnea, combined with a significant smoking history and family history of premature coronary artery disease, raise concern for possible stable angina. However, the absence of acute ECG changes suggests a lower likelihood of acute coronary syndrome at this time.

****Plan****

- **Medications:**

- Aspirin 75 mg daily (to be initiated)

- **Examinations:**

- Lipid profile (bilan lipide)

- **General Plan:**

- Smoking cessation counseling
- Adoption of a low-fat diet
- Regular walking exercise (30 minutes per day)

- **Follow-up:**

- Return in 3 days for review of blood results
- Routine follow-up scheduled for May 9, 2026